

Scintillations



“If Dogs could talk, perhaps we’d find it just as hard to get along with them as we do with people.”
-Karel Capek

Solav
PET CT



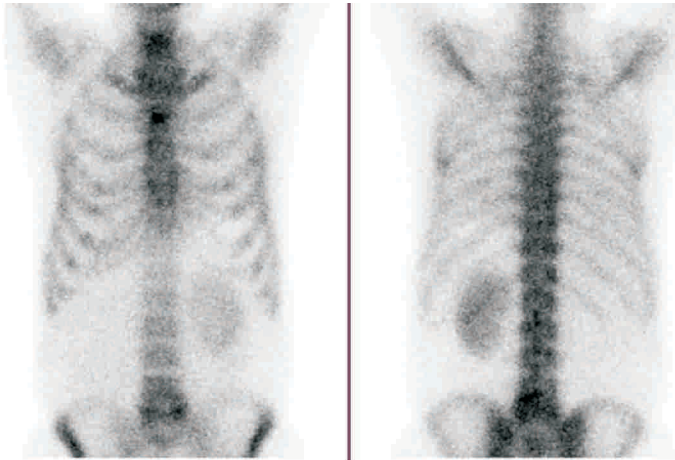
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Case18. Not all multifocal lesions in a known malignancy are metastatic



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Fig 18 A Bone scan showing features of degenerative changes



Fig 18 B MRI (T2)



Fig 18 B MRI (T1)

This 55 years old gentleman was operated for renal cell carcinoma 20 years ago. He presented with backache in Dec 08. A bone scan revealed degenerative changes in the lumbar vertebrae.(Fig 18A)
An MRI revealed abnormal signals in lower lumbar vertebrae. (Fig 18B)

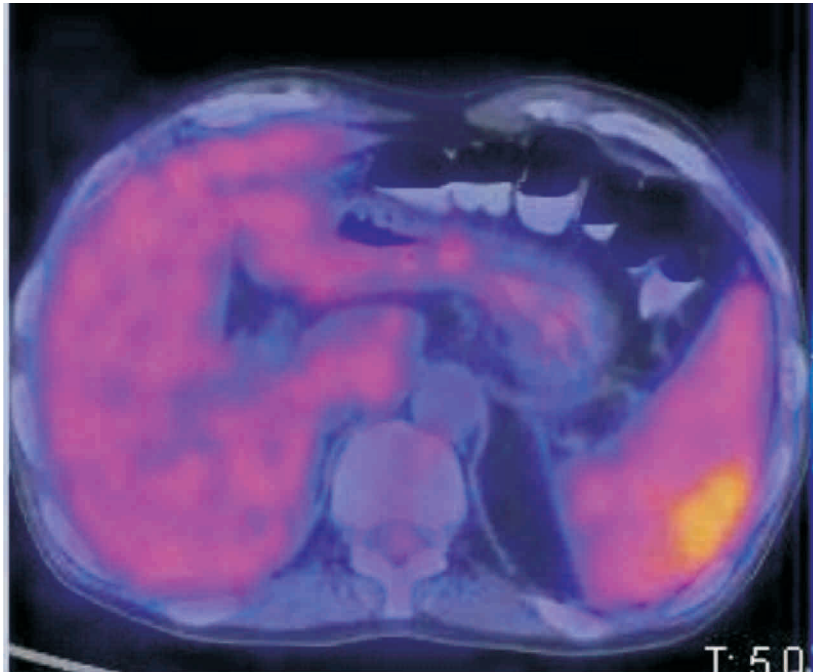


Fig 18 D Pre-treatment scan showing splenic lesion

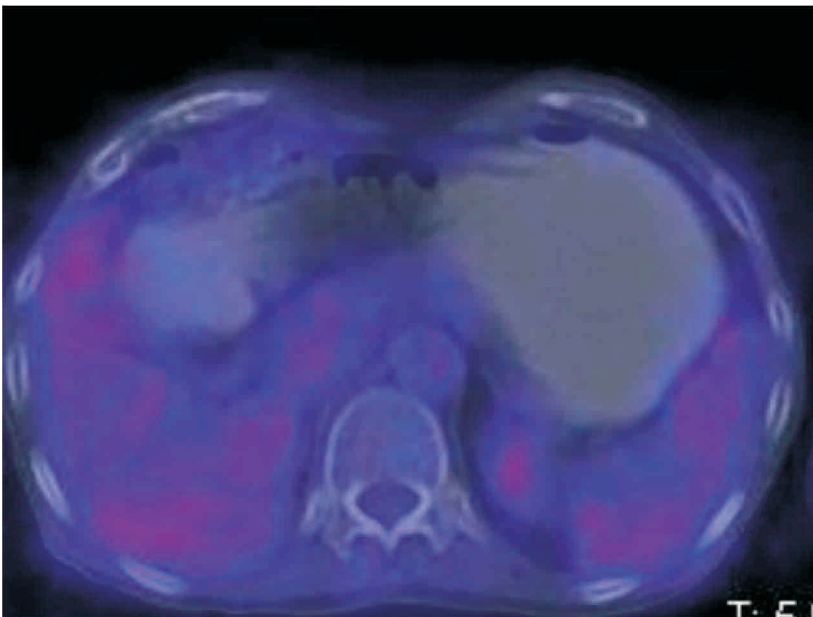


Fig 18 F 3monthspost Antitubercular Rx

While on antitubercular treatment, he showed clinical improvement and a follow up PET scan revealed disappearance of pulmonary and splenic (Fig 18F) lesions and regression in lumbar vertebral lesion. (Fig 18 G)

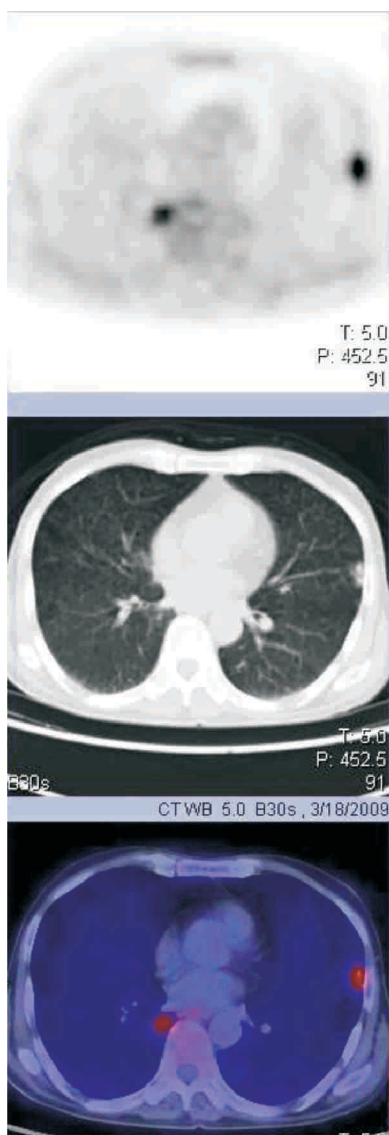


Fig 18 C Pre treatment scan showing Pulmonary lesions



Fig 18 E Pretreatment scan lumbar lesion

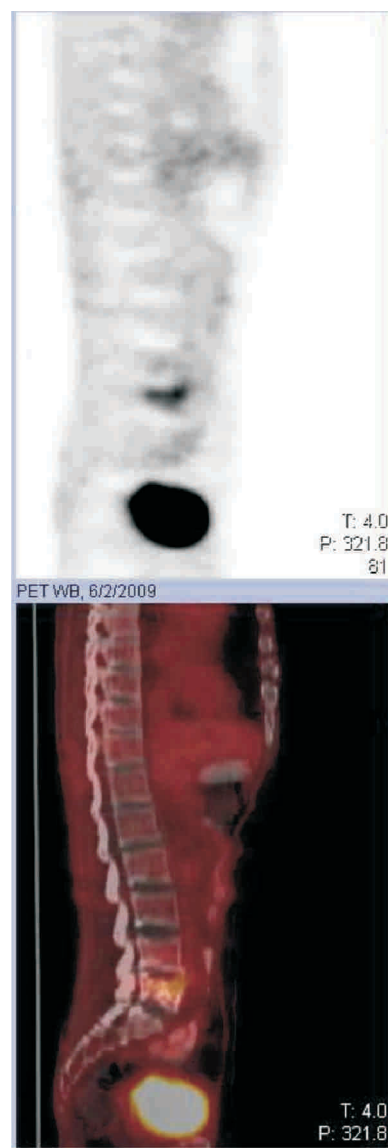


Fig 18 G 3months post AKT

In view of past history of malignancy and a negative bone scan, he was referred for a whole body PET scan. The test revealed multiple organ involvement viz. the lungs (Fig 18C), the spleen (Fig 18 D) and the bones at lumbar region. (Fig 18 E)
Histology discovered tubercular pathology.

Case19. Metabolically active lymph nodes should be biopsied for conclusion.

Contributed by : Dr AA Ranade, Dr Joshi, Dr Shrikant Solav

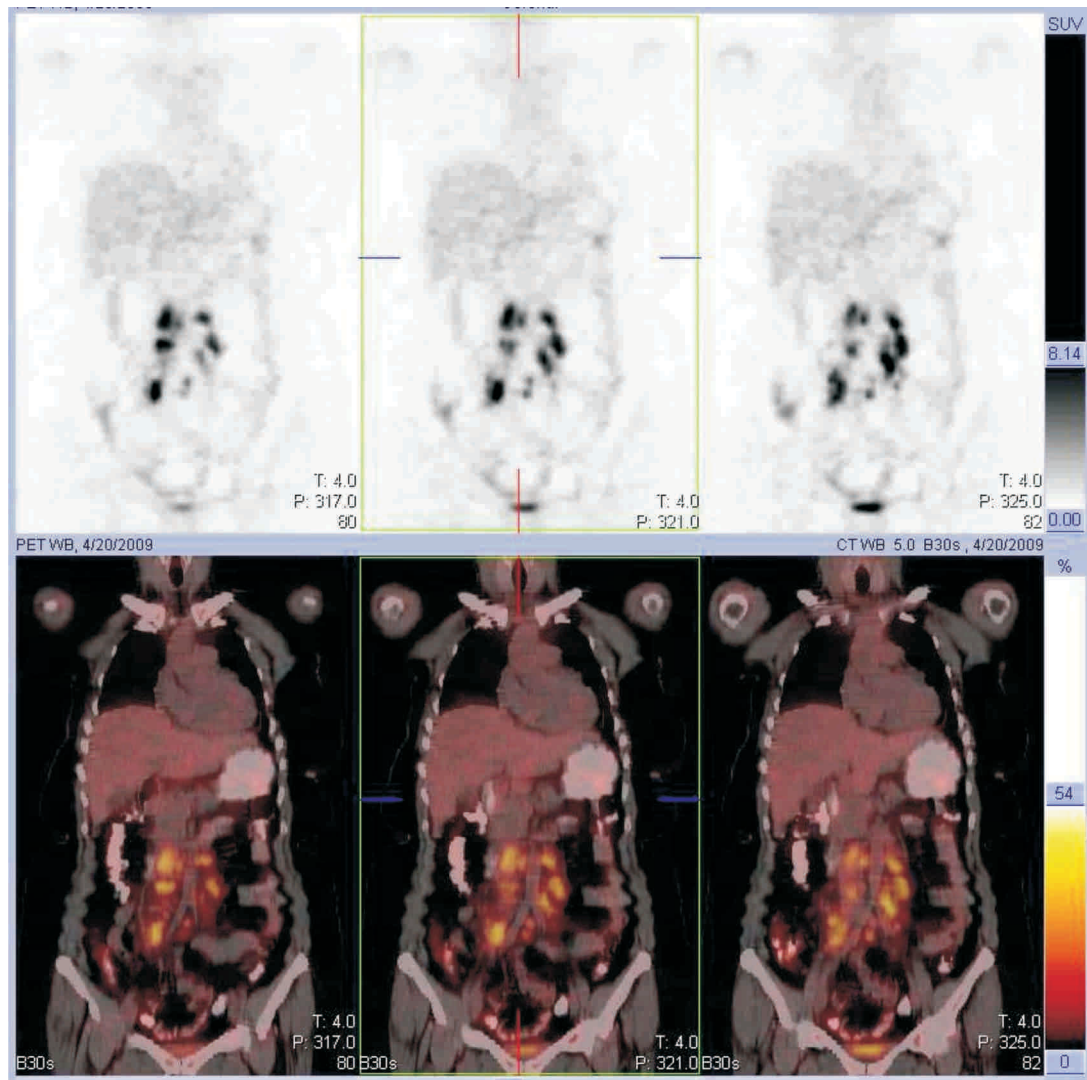


Fig 19 retroperitoneal lymphadenopathy coregistration with CT scan in coronal section

This 40 years old lady was operated for carcinoma of breast 5 years ago.

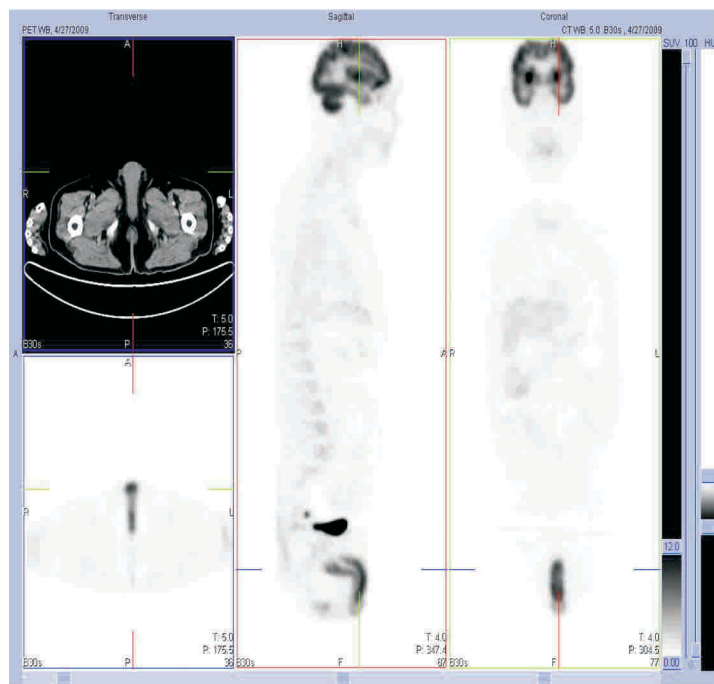
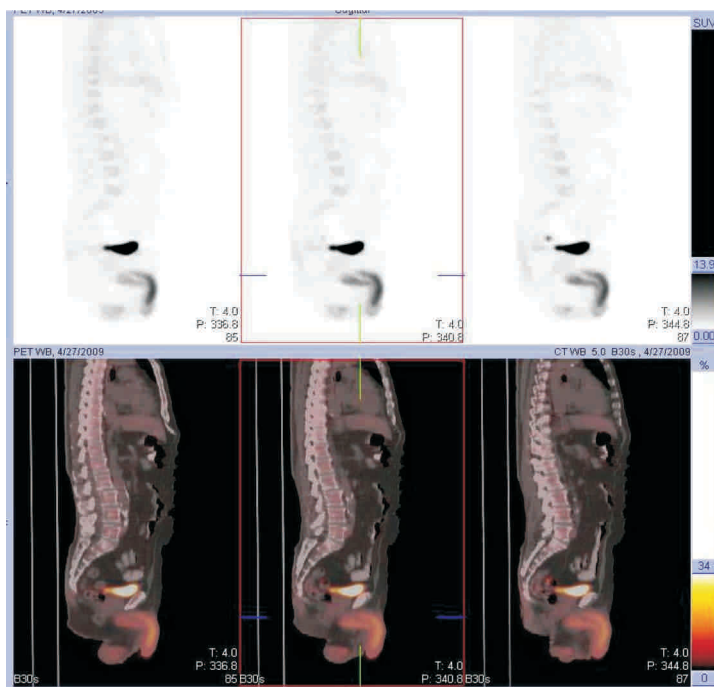
On a routine follow up sonography examination, she was found to be having retroperitoneal lymph nodal enlargement.

A PET CT was ordered for the purpose of restaging.

There were multiple metabolically active retroperitoneal lymph nodes as shown in Fig 19. Histologic evaluation was suggested to exclude the possibility of tuberculosis/ lymphoma. Histology revealed tubercular involvement of these nodes.

Case 20: A case of PUO

Contributed by : Dr Ajit Yadav, Dr Deepak Salunke, Dr Naresh Rao, Dr Belsare, Dr Shrikant Solav



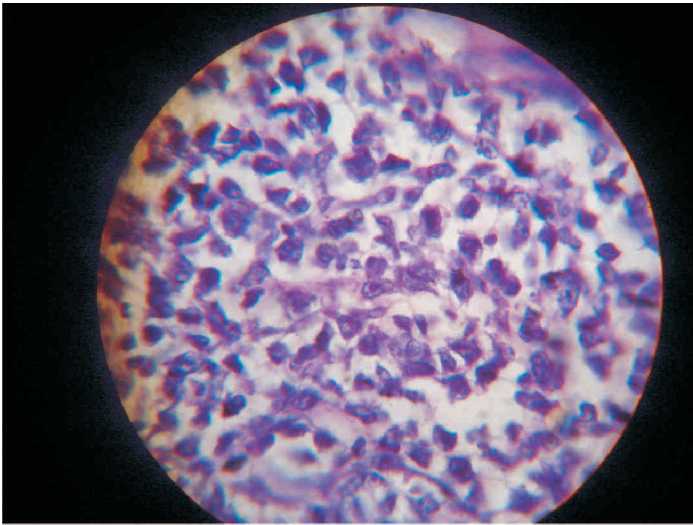


Fig 20 C Rt.cord NHL infiltrate-oil immersion.

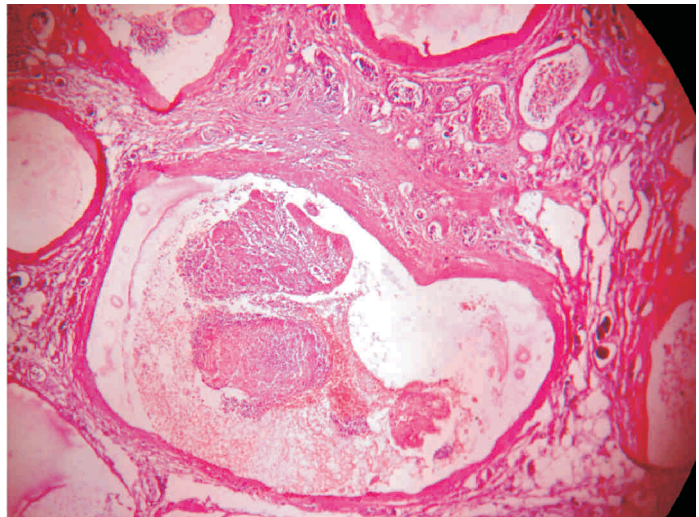


Fig 20 D Lt.cord NHL infiltrate-vascular and lymphatic invasion-HP

This 80 years old gentleman presented with fever and night sweats in March 09. All his routine investigations were normal. There was a past history of pyrexia in 1990 and 2004 that responded to antibiotics.

APET scan was ordered to look for any obvious focus of hidden infection.

The whole body scan revealed diffuse increased uptake in the penis. (Fig 18 A,B) Further evaluation was suggested.

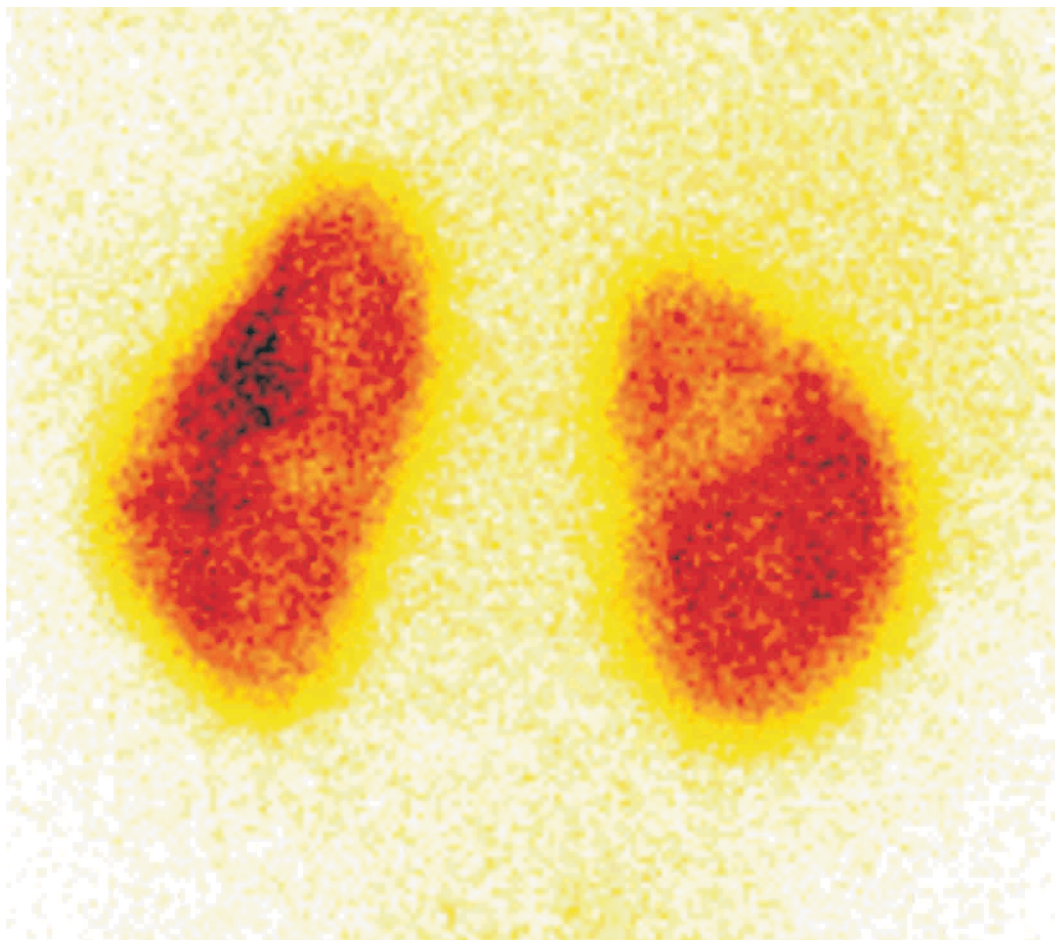
The gentleman continued to have intermittent fever.

On a follow up clinical examination, he was found to have tenderness in the perineum. Hence an ultrasound guided biopsy of the seminal vesicles was obtained.

The biopsy revealed Non Hodgkins lymphoma. (Fig 18 C,D)

Spot Case : DMSA scan in acute pyelonephritis

Contributed by : *Dr MR Bhalerao MD , Dr Shrikant Solav*



This 10 yrs old girl child presented with high grade fever and dysuria.

Clinical examination was unremarkable.

Urine examination revealed plenty of pus cells.

Sonography of abdomen was normal.

DMSA scan revealed cortical based photon deficiency in the superior pole of right kidney without volume loss of the kidney. The pattern was typical of acute pyelonephritis.

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